Stage 3: Braces off

Bone graft for a cleft in the gum: the three stages

This model shows the same jaw in the later teens, after the braces come off. Your dentist can now fill the gap where the small tooth never grew.

PLACEHOLDER: picture of the Braces off model

Implant with a crown

Render with tools/sheet/render.js once the model is built.

Implant with a crown

PLACEHOLDER: picture of the Braces off model

Bridge with one wing

Render with tools/sheet/render.js once the model is built.

Bridge with one wing

PLACEHOLDER: picture of the Braces off model

Plate with a false tooth

Render with tools/sheet/render.js once the model is built.

Plate with a false tooth

PLACEHOLDER: picture of the Braces off model

Left side: the eye tooth in the row of teeth

Render with tools/sheet/render.js once the model is built.

Left side: the eye tooth in the row of teeth

What has changed

The braces are off. Bone holds the teeth where the gap was.[1,2]

You have stopped growing, so your dentist can now fill the gap for good.[3,4]

Ways to fill the gap

There are three main ways:[3]

- an **implant**: a small post in the bone with a false tooth (a **crown**) on top[4]
- a **bridge**: a false tooth with one wing glued to the back of the tooth next to the gap
- a **plate you can take out** (a partial denture) that holds a false tooth

The pictures show each choice.

Some teams close the space instead. Braces move the eye tooth into the place of the missing tooth. Then the dentist reshapes it so it looks like the missing tooth. Teams use this plan most often for children with a cleft. The model does not show this plan.[5,1,6,7]

Your cleft team will talk with you about which plan suits you.[3]

Sources **1.** Bergland O, Semb G, Abyholm FE. Elimination of the residual alveolar cleft by secondary bone grafting and subsequent orthodontic treatment. Cleft Palate J. 1986;23:175-205. PMID 3524905. pubmed.ncbi.nlm.nih.gov/3524905 **2.** Jahanbin A, et al. Long-term stability of alveolar bone graft in cleft lip and palate: a systematic review and meta-analysis. J Craniofac Surg. 2022;33:e194-e200. doi.org/10.1097/SCS.0000000000008254 **3.** Clerc MM, et al. Replacement of missing lateral incisors for patients with cleft lip and palate: a decision-making tree based on a systematic review. J Prosthet Dent. 2025;134:50-60. doi.org/10.1016/j.prosdent.2023.07.023 **4.** Knobloch LA, et al. Narrow diameter implants for a missing lateral incisor in cleft palate patients: five-year results. J Prosthet Dent. 2025;134:1738-1747. doi.org/10.1016/j.prosdent.2024.07.048 **5.** Lee A, et al. Dental rehabilitation of the unilateral cleft dental gap: long-term follow-up. J Craniofac Surg. 2024;35:199-202. doi.org/10.1097/SCS.0000000000009802 **6.** Kokich VO Jr, Kinzer GA. Managing congenitally missing lateral incisors. Part I: canine substitution. J Esthet Restor Dent. 2005;17:5-10. doi.org/10.1111/j.1708-8240.2005.tb00076.x **7.** Zachrisson BU, Rosa M, Toreskog S. Congenitally missing maxillary lateral incisors: canine substitution. Am J Orthod Dentofacial Orthop. 2011;139:434-438. doi.org/10.1016/j.ajodo.2011.02.003

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